

PACKAGE INSERT

MYCOSONE CREAM

Composition: Hydrocortisone acetate 1.12% w/w
(equivalent to Hydrocortisone 1% w/w)
Miconazole Nitrate 2% w/w

Product Description:

White, water miscible cream.

Pharmacodynamics:

Hydrocortisone Acetate

Hydrocortisone Acetate is a natural corticosteroid with potent anti-inflammatory, anti-pruritic and vasoconstrictive actions. The mechanism of anti-inflammatory activity of the topical steroid is not clear. However, there is some evidence to suggest that a recognizable correlation exist between vasoconstrictor potency and therapeutic efficacy in man. Besides, it appears to act by stabilizing the membrane of lysosomes, thereby promoting the containment not only of cathepsins, but also of other enzymes within the lysosomal sac.

Miconazole Nitrate

Miconazole is an imidazole anti-fungal agent. It blocks the synthesis of ergosterol, which is the main sterol in the fungal cell membrane, by interacting with the enzyme necessary for the conversion of lanosterol to ergosterol. The resulting depletion of ergosterol alters the fluidity of the membrane and this interferes with the action of membrane associated enzymes. A further repercussion is the inhibition of the transformation of candidal yeast cells into hyphae (the invasive and pathogenic form of the parasite). It is fungistatic and has a wide spectrum of anti-microbial activity in vitro including activity against *Candida spp.*, *Blastomyces dermatitidis*, *Coccidioides immitis*, *Cryptococcus neoformans*, *Epidermophyton floccosum*, *Histoplasma capsulatum*, *Paracoccidioides brasiliensis*, *Malassezia furfur*, *Microsporum canis*, *Trichophyton mentagrophytes*, *T.Rubrum*, *Cladosporium*, *Madurella*, *Phialophora spp.* and *Pseudallescheria boydii*. Besides, it also possesses some antibacterial activity against gram-positive bacteria including some staphylococci and streptococci.

Pharmacokinetics:

Hydrocortisone Acetate

The extent of percutaneous absorption of topical corticosteroids is determined by several factors, such as the vehicle, the integrity of the epidermal barrier, and the use of occlusive dressings. Depending on the location and type of infection, hydrocortisone penetrates well into the skin and is partially absorbed. More than 90% of the absorbed fraction is bound to plasma proteins. The metabolism takes place in the liver and tissues and the metabolites are excreted in the urine, mostly as glucuronides, together with a very small fraction of unchanged hydrocortisone.

Miconazole Nitrate

Miconazole does not produce detectable blood levels after topical application. Although most of the applied drug remains on the skin surface, drug concentrations were found in the stratum corneum, which by far, exceeded the minimum inhibitory concentration for dermatophytes. Inhibitory concentrations were achieved in the epidermis and as deep as the middle region of the dermis.

Indication:

Mycosone Cream is indicated for infection of the skin by dermatophytes or *Candida sp.* with inflammatory symptoms. Thus, Mycosone Cream is particularly indicated for the initial stages of treatment. Once the inflammatory symptoms have disappeared, treatment may be continued with Mycosone Cream which is preferred.

Recommended Dosage:

Apply the cream topically to the lesion once or twice daily by rubbing the cream gently into the affected area until it is completely absorbed by the skin. Cultural isolation of the infecting organism should be followed by appropriate local or systemic antimicrobial therapy if no improvement is observed after 7 days.

Route of Administration: Topical.

Contraindications:

Mycosone Cream is contraindicated in those patient who have shown hypersensitivity to hydrocortisone acetate, miconazole nitrate and other ingredients in the preparation. Topical corticosteroids preparations are contraindicated in the treatment to tuberculous skin affections, herpes simplex, vaccinia and all forms of varicella.

Warnings and Precautions:

Long term continuous treatment with topical steroids should be avoided as far as possible as this may cause atrophic changes in the skin leading to thinning, loss of elasticity, dilatation of superficial blood vessels, telangiectasia and ecchymoses.

The changes are particularly likely to occur on the face and when occlusive dressings are used.

Systemic absorption of topically applied corticosteroids may occur when large quantities are used, or when application is made to wide areas of the body, or to damaged skin; and when the occlusive dressing technique is applied. Systemic absorption of topical corticosteroids has produced reversible hypothalamic-pituitary-adrenal (HPA) axis suppression, manifestations of Cushing's syndrome, hyperglycemia and glycosuria in some patients. Therefore, if HPA axis suppression is noted, an attempt should be made to withdraw the drug, to reduce the frequency of application, or to substitute with a less potent steroid.

In patients on warfarin, caution should be exercised and the anticoagulant effect should be monitored (see Interactions).

Interactions with Other Medicaments:

Miconazole administered systemically is known to inhibit CYP2C9 enzyme system. Due to the limited systemic availability after topical application, clinically relevant interactions occur very rarely. In patients on warfarin which is subjected to metabolism by CYP2C9, caution should be exercised and the anticoagulant effect should be monitored (see Warnings and Precautions).

Pregnancy and Lactation:

Miconazole administered systemically has been shown to be teratogenic after dermal application in animal studies. Therefore, topical corticosteroids should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. Drugs of this class should not be used extensively on pregnant patients, in large amounts, or for prolonged periods of time.

It is not known whether topical administration of corticosteroids could result in sufficient systemic absorption to produce detectable quantities in breastmilk. Systemically administered corticosteroids are secreted into breastmilk in quantities not likely to have a deleterious effect on the infant. Nevertheless, caution should be exercised when topical corticosteroids are prescribed for nursing woman.

Side Effects:

Local treatment is well tolerated. However, reports of burning, itching, irritation, dryness, folliculitis, hypertrichosis, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, skin atrophy, striae and milaria have been associated with the use of topical dermatologic corticosteroids, especially under occlusive dressings. Systemic absorption of topical corticosteroids has produced reversible hypothalamic pituitary adrenal axis suppression, manifestation of Cushing's syndrome, hyperglycemia and glucosuria in some patient. Besides, occasional burning and itching due to miconazole have been reported as well.

Symptoms and Treatment of Overdose:

If accidental ingestion of large quantities of the product occurs, an appropriate method of gastric emptying may be used if considered necessary. Prolonged and excessive use can results in skin irritation, which usually disappears after discontinuation of therapy.

Topically applied, corticosteroids can be absorbed in sufficient amounts to produce systemic effects.

Effects on Ability to Drive and Use Machine: Not applicable.

Storage Conditions: Store below 30°C.

Pack Sizes: A tube of 15g and 30g.

Shelf-life: 3 years.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:

Sunward Pharmaceutical Sdn. Bhd.

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EL215A-R5
Revision Date: 22/07/2025